Northern Ireland Hospice

CLINICAL POLICY

Missing Patient in Adult Services

Title	Missing Patient in Adult Services	**Policy number**	CSP21/2023
Original author	Faye Townley, Ward Manager	**Supersedes**	CSP24/2019
Updated by	Dr Matthew Doré	**Issue date**	November 2026
Approved by	Debbie Burns, Director of Care & Quality Performance	**Review date**	November 2036

NEW • INTERACTIVE TRAINING

Think you know what to do? Prove it in Code Missing

Code Missing is our interactive training game built around this policy. Race the clock through a missing patient scenario, make the calls that matter, and see how your score stacks up against your colleagues. It takes a few minutes, works on any phone or computer, and is the quickest way to make this procedure second nature before you ever need it.

▶️ **PLAY NOW: https://mattsharondore.github.io/code-missing/**

Ideal for induction, team huddles and annual refreshers. The game is a training tool: in a real situation, always follow this approved policy.

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Rationale

Northern Ireland Hospice recognises it has a fundamental duty to care for and protect its patients at all times. A patient may leave the hospice premises, often under the supervision of a member of staff or a relative. Patients who are confused by the environment can sometimes wander out of the hospice premises while looking for a relative, the bathroom or the kitchen, or simply because of their confusion, and then not find their way back.

Objectives

- To minimise the risk of a patient going missing by ensuring adequate controls are in place.
- To make staff and volunteers aware of the immediate and subsequent steps to be taken as soon as a patient is noticed missing.
- To ensure relevant senior staff, security and next of kin are informed, as set out in the Missing Patient Record (Appendix 2).

Policy

- On admission to the service, each patient must be assessed to identify whether they are confused or at risk of wandering. If they are at risk, consideration must be given to the location of the patient's room and to care plan management, so that their safety is maintained at all times and they do not leave the building unaccompanied.
- Consider whether a Deprivation of Liberty (DoL) is appropriate, and discuss with the wider MDT whether there is a need to implement the DoL protocol.
- The clinical team must tell relevant non-clinical staff and volunteers that the patient is at risk of wandering. This may be done verbally or through the volunteers' diaries.
- Once a patient is noticed missing in any area, whether the in-patient unit or day hospice, an organised search must begin **immediately** to locate them. The patient's mental and physical ability must be established first by liaising with the relevant clinical staff.
- The missing patient procedure must then be started to support locating the patient (see Appendix 1 and Appendix 5).
- All reporting mechanisms must be put in place as per the procedure.
- The Major Incident Plan must be activated.
- Once the patient is located, all staff involved must be informed to stand down.
- Any patient wearing pyjamas or inappropriate attire and seen leaving the hospice premises should be intercepted by any member of staff or volunteer, and the IPU nursing team contacted immediately to help return the patient to the IPU.
- A report must be prepared for the Corporate Governance Safety Committee and an IR1 completed.
- This policy is supported by, and should be read in conjunction with, the Missing Patient Quick Action Card (Appendix 1) and the Missing Patient Procedure (Appendix 5).
- In accordance with the Independent Health Care Regulations (Northern Ireland) 2005, regulation 28, any event in the establishment which adversely affects the wellbeing or safety of any service user must be reported to the Regulation and Quality Improvement Authority (RQIA) within 24 hours.

Appendices

Appendix	Title
Appendix 1	Missing Patient Quick Action Card
Appendix 2	Missing Patient Record
Appendix 3	Herbert Protocol
Appendix 4	Missing Patient Search Plan (flow diagram)
Appendix 5	Missing Patient Procedure



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Appendix 1: Missing Patient Quick Action Card

Keep this card visible on the ward. Colours match the phases in the Search Plan (Appendix 4).

When	Action	Who
Immediately	Tell the Nurse in Charge. Note when and where the patient was last seen and what they were wearing.	Anyone
0–10 min	Open the Missing Patient Record (Appendix 2). Assess risk with the doctor. **HIGH RISK → call 999 NOW.**	NIC + doctor
0–10 min	Search the immediate area and the adjoining garden. Ask patients and visitors. Try the patient's phone. Check whether family have taken them out.	NIC allocates staff
By 15 min	Escalate: IPU Manager (9am–5pm) or Clinical Manager on call → Senior Manager on call.	NIC
By 15 min	Search all departments and grounds using the Search Plan (Appendix 4). Each area reports back with name, role and time.	All departments
By 15 min	Check live and recorded CCTV. Preserve the footage. Search car parks and perimeter.	Security
By 30 min	**Call 999** (if not already done). Give the description, Herbert Protocol form/photo (Appendix 3), risks and likely destinations. Name a police liaison. Record the reference number.	Senior Manager / NIC
By 30 min	Inform the next of kin. Ask them to check home and familiar places. Agree how they will be updated.	Senior Manager / NIC
As needed	Decide whether to activate the Major Incident Plan.	Senior Manager
When found	Make the patient safe. Clinical review. Inform police, next of kin and all areas. **Stand down.**	NIC / Senior Manager
Same shift	IR1 incident report + Missing Patient Record. Document in the health record. Review the care plan. Debrief staff.	NIC
Within 24 h	Notify RQIA through the Web Portal (regulation 28). Consider adult safeguarding and SAI/patient safety incident review.	Registered Manager

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Appendix 2: Missing Patient Record

Start this record as soon as a patient is reported missing. Record all times in the 24-hour clock. Attach it to the IR1.

Section A: Patient details			
Patient name		**Hospice number**	
Date of birth / age		**Ward / department**	
Diagnosis		**Herbert Protocol form / photo held? (Appendix 3)**	Yes ☐ No ☐
Key risks			
Last seen (time and place)			
Clothing / description			
Time noticed missing		**Reported by (name / role)**	
Risk level	HIGH ☐ Not high ☐	Assessed by: _____	Time: _____

Section B: Search and escalation			
Action	**Done?**	**By (name / role)**	**Time**
Immediate area searched	Found ☐ Not found ☐		
Patients / visitors asked; phone tried; family checked	☐		
IPU Manager / Clinical Manager on call informed	☐		
Senior Manager on call informed	☐		
All departments and grounds searched (Appendix 4)	☐		
Security informed; CCTV reviewed and preserved	☐		
PSNI informed (999). Ref no: _____	☐		
Next of kin informed	☐		
Major Incident Plan activated	Yes ☐ No ☐		

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Section C: Outcome			
Action	**Done?**	**By (name / role)**	**Time**
Patient found (where): _____	☐		
Clinical review completed	☐		
PSNI, next of kin and all areas informed; stand-down confirmed	☐		
Entry made in health record	☐		
IR1 completed. IR1 no: _____	☐		
RQIA notified via Web Portal (within 24 h)	☐		
Safeguarding considered / referral made	☐		
Care plan, risk and capacity/DoLS reviewed	☐		
Staff debrief held	☐		

Completed by		**Role**	
Signature		**Date**	

Send the completed record with the IR1 to the Clinical Governance and Safety Committee.

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Appendix 3: Herbert Protocol

A Northern Ireland scheme that helps police find a person living with dementia quickly if they go missing.

What it is

The Herbert Protocol is a simple risk-reduction tool launched by the Police Service of Northern Ireland (PSNI) together with Dementia NI, the local Health and Social Care Boards and the Policing and Community Safety Partnerships. It is a form that is filled in ahead of time, holding key information about a person living with dementia.

What the form records

Information	Why it helps
Medication	Tells police and responders about clinical risk and urgency.
Description	Gives an accurate physical description to circulate.
Photograph	A recent photo can be shared with search teams straight away.
Significant places	Former homes, workplaces and favourite places the person may head for.
Daily routine	Habits and patterns that suggest where the person might go.

How the form is kept and used

- It is completed in advance and stored safely, either electronically or as a printed copy.
- It must be easy to find at any time of day or night, so whoever starts the initial search can hand it over straight away.
- Police will only ask for the form, or the information in it, if the person is reported missing. Its purpose is to help police and other agencies find the person quickly and return them to safety.

How it links to this policy

- Section A of the Missing Patient Record (Appendix 2) asks whether a **Herbert Protocol form / photo is held** for the patient.
- When calling 999, give police the description, the **Herbert Protocol form or photo**, key risks and likely destinations (Appendix 1, “By 30 min”).

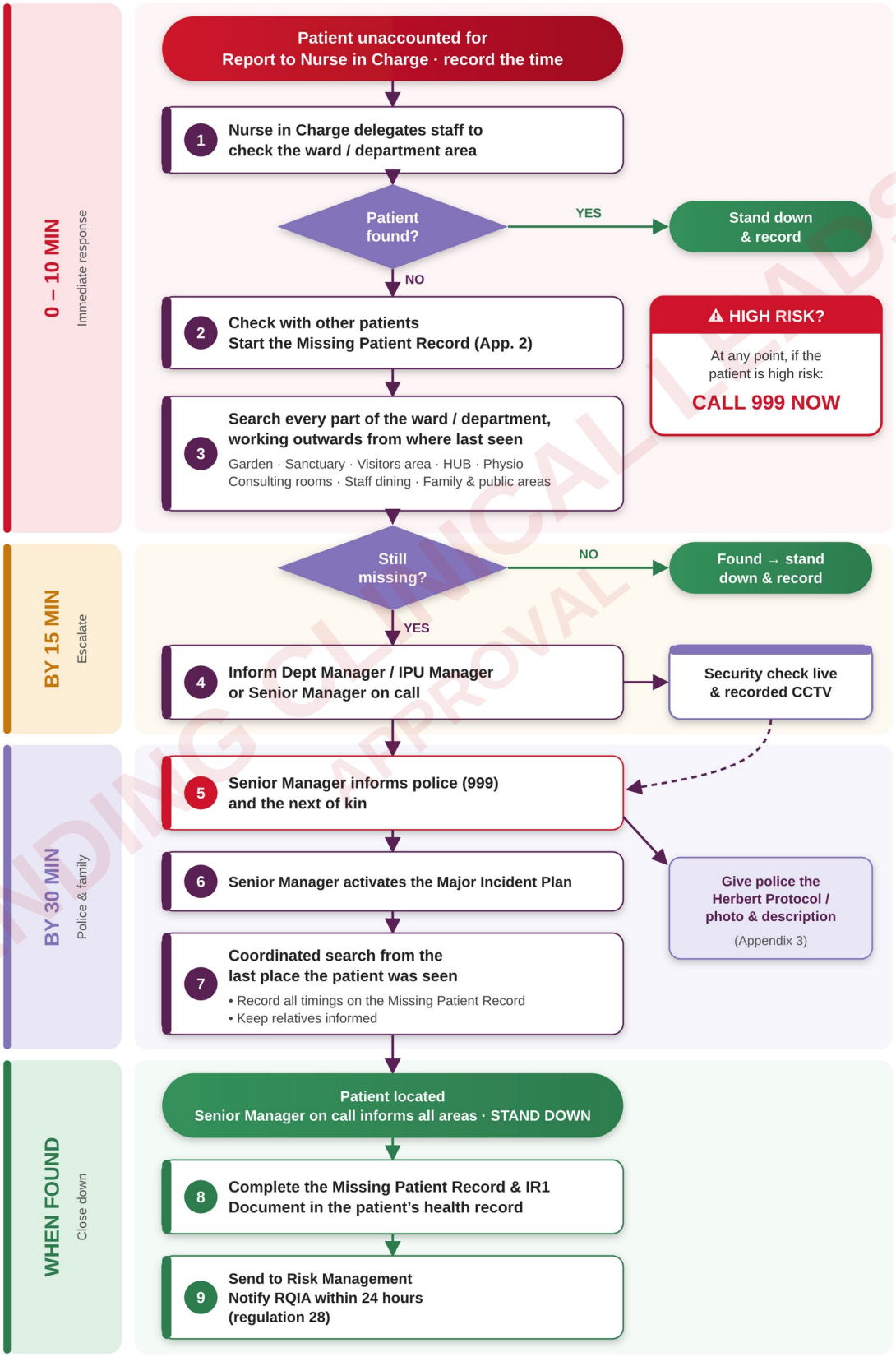
Further information and downloads

The Herbert Protocol form and information leaflet can be downloaded from the PSNI website:
https://www.psni.police.uk/safety-and-support/advice-and-information/missing-persons/herbert-protocol

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Appendix 4: Missing Patient Search Plan

Follow the flow from the top. Coloured bands show the target timings in the Quick Action Card (Appendix 1).



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Appendix 5: Missing Patient Procedure

The full procedure behind the Quick Action Card and Search Plan.

Rationale

When a patient is not accounted for and there is no explanation as to their whereabouts, this procedure must be followed to ensure effective communication with staff, managers, relatives and the police in order to locate the missing patient.

Procedure

1. The member of staff or volunteer who notices the patient is missing must report to the Nurse in Charge or head of department. If the patient was attending another hospice department (for example the Hospice Hub or the Sanctuary), the ward must be informed. A Missing Patient Record (Appendix 2) must be started to document all actions taken.
2. The patient's Consultant or doctor must be consulted to establish the key problems and the level of risk.
3. All staff must react promptly and carry out their designated duties without delay.
4. Search the immediate area using the outline drawing and checklist for that area. Keys for locked areas must be made available; the IPU Manager, team leaders and security hold master keys. Where possible, ask patients in the immediate area whether they saw where the patient went.
5. If the patient has still not been found, the Nurse in Charge must contact the IPU Manager (9am–5pm) or, out of hours, the Clinical Manager on call, as per the Major Incident Plan.
6. The Clinical Manager must be informed once the search of the immediate area has been completed, and when the patient has been found.
7. Record all times and actions on the Missing Patient Record.
8. Widen the search to surrounding departments, each using its own outline plan and checklist (see the Search Plan, Appendix 4). Each department reports back on completion, giving name, designation and the time the search was completed.
9. Security staff should be asked to view live pictures and any available video recordings for the immediate and surrounding areas.
10. Once satisfied that the patient is not on hospice premises, the Senior Manager will contact the PSNI to advise them of the situation and put them on standby. The Senior Manager should initiate the Major Incident Plan.
11. The PSNI should be informed once an initial search of the local area is completed and the patient is not found, or if the patient is seen on a video recording at an exit. Provide the Herbert Protocol form or photo if one is held (Appendix 3).
12. The patient's next of kin should be contacted, as patients often try to make their way home.
13. Record all times, actions and the individuals involved on the Missing Patient Record.
14. Once the patient is found, ward staff should inform the Nurse in Charge and the Senior Manager, who is responsible for informing all areas so they can stand down.
15. An IR1 report, accompanied by the Missing Patient Record, must be sent to the Safety Committee.
16. All missing patient incidents and near misses must be reported using the IR1 form, following the accidents, incidents and near miss procedure.
17. The Regulation and Quality Improvement Authority (RQIA) must be informed within 24 hours of the event. This must be done by the head of department.

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